



Best Practice Prescription Program Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Mental Health Service	Mental Health Services	Medical, Pharmacy, Nursing, Clerical

2. Purpose

This document aims to inform AHS staff of personnel roles and responsibilities relating to the use and maintenance of the Best Practice Prescription Program.

3. Background

Best Practice Prescription Program is an electronic program implemented in the AHS outpatient mental health clinics that is used for the following activities:

- Prescription generation (replacing handwritten prescriptions*)
- Maintenance of electronic medication profiles (replacing paper-based profiles)
- Internal and external correspondence generation
- Recording other relevant clinical information (e.g.: allergies/ adverse drug reactions).

Best Practice Prescription Program is to be used to generate prescriptions for consumers who attend the AHS outpatient mental health clinics.

Mental Health Inpatient services contact, Consultation Liaison contact or ED contact for mental health **must** update the medication profile in Best Practice at the time of discharge from their respective services to avoid errors.

Prescriptions supplied for patients being discharged from inpatient mental health wards are to be generated from the Notifications & Clinical Summaries (**NaCS**) program.

4. Procedure

4.1 Creating patient entries in Best Practice Prescription Program:

- Clinic clerks are responsible for entering consumer demographic data to create a consumer profile for each existing and newly accepted patient of that clinic and activated on PSOLIS.
- The minimal demographic data required is the consumer's:
 - Full name
 - Date of birth
 - Address
 - Unit medical record number (UMRN)

- Medicare and Concession numbers (if known)
- Where changes (e.g.: merging of consumer profiles) occur in other hospital systems (e.g.: WebPAS), this information is communicated to the PSOLIS Coordinator.
- The PSOLIS Coordinator will forward this information to the Clerical Coordinator (Mental Health), who will be responsible for forwarding this information onto the clerical staff and to the Best Practice Prescription Program Administrator (Administration staff at clinics) for merging.
- The Best Practice Prescription Program Administrator must ensure the above changes are updated in Best Practice Prescription Program as soon as possible.

4.2 Maintaining a medication profile in Best Practice

Prescription Program Appointment letters sent to consumers prior to their scheduled appointment will contain a reminder for all medications (psychiatric and non-psychiatric) to be brought to each appointment to facilitate history taking and updating of medication profiles. (refer to Appendix 1)

The prescriber is responsible for the following:

- At the first clinic appointment, medical officers are to inquire with the consumer (or carer) about their current medications and the medical officer or care co-ordinator may need to verify this information with the consumers own medications or a suitable alternative source (e.g.: General Practitioner).
- The details (i.e.: medication name, formulation, route, dose and directions for use) of each psychiatric and non-psychiatric medication must be entered into the Best Practice Prescription Program medication profile.
- At each subsequent appointment at any of the outpatient mental health clinics (inclusive of the discharge clinic) or presentation to the Armadale Mental health inpatient services, Armadale Hospital Emergency Department, Consultation Liaison service; the prescriber / medical officer must inquire about medication changes and update the profile in the Best Practice prescription program prior to discharge .
- When updating profile details, prescribers must ensure that any superseded information is removed.
- The prescriber must inquire about the consumer's allergies/ adverse drug reactions (if any) and document these details (offending agent and reaction) in Best Practice. Prescription Program as per [AKG Alerts – Identification and Documentation](#) This information must be reviewed at each clinic attendance and updated in Best Practice Prescription Program, when required.

4.3 Generating prescriptions from Best Practice Prescription Program

- Best Practice Prescription Program is to be used for generating prescriptions for all medications prescribed in the outpatient mental health clinics.
- Downtime procedure:
 - *Where Best Practice Prescription Program is not available (e.g.: power outage, home visits), prescribers are to consult the Clinic Coordinator for a handwritten prescription pad.
- For each Best Practice Prescription Program prescription generated, 2 copies are to be printed:

- A signed patient copy to be given to the patient.
- A signed 2nd copy (stamped “File Copy”) to be filed in the patient’s medical record.
- For each Best Practice Prescription Program prescription generated, a signed copy should be provided to the patient. If the script is being faxed to the community pharmacy, a signed copy of the script should not be provided to the consumer due to potential risk of duplicate dispensing.
- Where changes to the consumer’s usual medication regime have been made, the completed medication sticker is to be inserted into the medical record and the consumer informed as per [WA Health Mental Health Consumer Medication Information Policy OD 0069/17](#) with a brief description to be entered in the “Comments” field of Best Practice Prescription Program (e.g.: dose increase of 25mg). This is to allow better communication of changes to other healthcare providers (e.g.: dispensing pharmacist).

4.4 Letter generation from Best Practice

- Letter templates (example shown in Appendix 1) can be incorporated into Best Practice Prescription Program. Prescribers are to utilise the Best Practice Prescription Program templates for communicating the patient’s management and progress to other relevant healthcare providers (e.g.: General Practitioner).
- Refer to Appendix 2 for a flowchart of the Best Practice Prescription Program process.

5. Education

The following positions have been identified as Best Practice Prescription Program ‘champions’, who will serve as trainers for new staff within their relevant discipline:

Position	Responsible for training the following personnel
CTT Consultant Psychiatrist	CTT Medical staff
ATT Consultant Psychiatrist	ATT Medical staff and inpatient medical staff, ED DMO and CL team
Older Adult Consultant Psychiatrist	Older Adult Medical staff
Clinic Coordinator	Nursing/Allied Health
CTT Clinical Nurse Specialist	CTT Nursing staff
ATT Clinical Nurse Specialist	ATT Nursing staff
Clerical Coordinator	Clerical staff
Mental Health Pharmacist	Pharmacy staff

6. New clinic prescribers

As prescribers are issued with unique logins on an annual subscription basis, the Administrative Assistant to the Mental Health Service Director is responsible for informing the Mental Health

Pharmacist of all medical staffing changes. The Mental Health Pharmacist will inform Graylands Hospital of these changes so that access can be updated, as appropriate.

7. Legislative context

The following documents are required to give effect to this procedure:

- *Medicines and Poisons Act 2014*
- *Medicines and Poisons Regulations 2016*
- *Mental Health Act 2014*
- Health Department of Western Australia (HDWA) [Clinical Handover Policy MP 0095](#)
- HDWA [Mental Health Consumer Medication Information Policy MP 0069/17](#)
- HDWA [Clinical Alert \(MedAlert\) Policy MP 0053/17](#)
- East Metropolitan Health Service (EMHS) [Patient Identification and Procedure Matching EMHS: 21](#)

8. Supporting information

The following documents support the implementation of this procedure:

- [Medication AKG-POL-0253-13](#)
- [Medication Management Procedure AKG-PRO-0255-18](#)

9. Compliance, monitoring and evaluation

Compliance with this procedure will be monitored by the members of the Mental Health Safety and Quality meeting.

Monitoring activities include:

- Quarterly auditing of medical records and Best Practice Prescription Program profiles to identify if staff have complied with history taking and record keeping requirements, as outlined in this procedure.
- Monthly monitoring of DATIX CIMS reports by the AHS Drugs and Therapeutics Committee and Mental health Safety and Quality Committee to identify any errors related to the change in prescribing program.

10. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 4 – Action 4.3 Partnering with consumers
- Clinicians use organisational processes from the Partnering with Consumers Standard in medication management to:
 - a. Actively involve patients in their own care
 - b. Meet the patient's information needs
 - c. Share decision-making
- Standard 4 – Action 4.5 Medication reconciliation
- Clinicians take a best possible medication history, which is documented in the healthcare record on presentation or as early as possible in the episode of care

- Standard 4 – Action 4.6
- Clinicians review a patient's current medication orders against their best possible medication history and the documented treatment plan, and reconcile any discrepancies on presentation and at transitions of care
- Standard 4 – Action 4.10 Medication review
- The health service organisation has processes:
 - a. To perform medication reviews for patients, in line with evidence and best practice
 - b. To prioritise medication reviews, based on a patient's clinical needs and minimising the risk of medication-related problems
 - c. That specify the requirements for documentation of medication reviews, including actions taken as a result
- Standard 4 – Action 4.11 Information for patients
- The health service organisation has processes to support clinicians to provide patients with information about their individual medicines needs and risks
- Standard 4 – Action 4.12 Provision of a medicines list
- The health service organisation has processes to:
 - a. Generate a current medicines list and the reasons for any changes
 - b. Distribute the current medicines list to receiving clinicians at transitions of care
 - c. Provide patients on discharge with a current medicines list and the reasons for any change

11. Policy owner (relating to these procedures)

Enquiries relating to this procedure may be directed to:

Title: Service Director
 Department: Mental Health
 Email: karen.elliott@health.wa.gov.au

12. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3.0	06/03/2025	06/03/2028	Scheduled review

13. Authorisation

This procedure has been authorised and issued by the Service Director Mental Health.

Authorised by	Karen Elliott – Service Director Mental Health
Authorisation date	05/03/2025
Published date	06/03/2025

Appendix 1: Letter template



Government of Western Australia
East Metropolitan Health Service
Armada Kalamunda Group



Date XX/XX/XXXX

Dear Doctor XXXX

RE: Patient XX (UMRN XXXXXX), DOB XX/XX/XXXX

I recently reviewed the above patient at the XX Clinic on date XX/XX/XXXX.

Current medications (as of date XX/XX/XXXX)

Medication	Dose	Indication

Patient's progress

Please contact me if you have any questions.

Yours sincerely,

Dr XX

Appendix 2: Process flowchart

